UP-11 Pain Control - Adult

MEDICAL-DIRECTOR APPROVED APP CONTENT | AGENCY RELEASE PENDING | Reconciled October 10, 2026

Scope: Use for patients age 16 years or older. For patients age 2 through younger than 16 years, use PM-04 Pediatric Pain Control. For patients younger than 2 years, contact Medical Control for analgesic direction.

Purpose and quick flow

1. Assess	Pain score, cause, allergies, previous analgesics, airway, breathing, perfusion, mental status, and medication-specific risk.
2. Treat cause	Address immediate threats first: airway care, hemorrhage control, splinting, positioning, ice when appropriate, and reassurance.
3. Select route	Mild pain with safe oral route: one oral nonopioid. Parenteral nonopioid: AEMT or Paramedic. Moderate-severe pain: Paramedic opioid or analgesic-dose ketamine.
4. Monitor	Cardiac monitoring for systemic nonoral analgesia. Continuous SpO2 and waveform EtCO2 with opioid or ketamine. Reassess every 5-10 minutes.
5. Transport	Do not delay airway management, hemorrhage control, splinting, or time-sensitive transport to complete analgesia or reach a target pain score.

Standing-order oral and parenteral nonopioids

Medication	Dose / route	Key safeguard
Acetaminophen - oral	EMT/AEMT/Paramedic: 650-1,000 mg PO once.	Use only when the patient protects the airway and can swallow safely. Do not give with severe liver disease/failure, suspected overdose, recent maximum dose, or uncertain prior dose/time.
Ibuprofen - oral	EMT/AEMT/Paramedic: 400 mg PO once.	Do not give with NSAID contraindications or recent NSAID administration. Do not combine with ketorolac.
Acetaminophen - IV/IO	AEMT/Paramedic: 1 g IV/IO once.	Include pre-EMS acetaminophen when screening for a recent maximum dose. Do not routinely add an oral acetaminophen dose.
Ketorolac	AEMT/Paramedic: 15-30 mg IV once OR 60 mg IM once.	Do not give with NSAID contraindications. Do not combine with ibuprofen or another NSAID.

NSAID exclusions: hypersensitivity, renal disease or transplant, significant dehydration, hypotension, active GI bleeding, clinically significant anticoagulant-associated bleeding risk, known or suspected pregnancy, or recent NSAID administration.

UP-11 Pain Control - Adult

PARAMEDIC ANALGESIA | Select one opioid OR analgesic-dose ketamine

Paramedic opioid options

Medication	Dose / route	Key safeguard
Fentanyl	50 mcg slow IV/IO; repeat 25-50 mcg every 5 minutes as needed; maximum total 100 mcg.	Use continuous SpO2 and waveform EtCO2. Do not combine with morphine or hydromorphone.
Morphine	5 mg slow IV/IO; repeat 5 mg after 10 minutes as needed; maximum total 10 mg. If no vascular access: 5-10 mg IM once.	Use continuous SpO2 and waveform EtCO2. Do not combine with fentanyl or hydromorphone.
Hydromorphone	0.5 mg slow IV/IO; repeat 0.5 mg after 10 minutes as needed; maximum total 1 mg. If no vascular access: 0.5-1 mg IM once.	Use continuous SpO2 and waveform EtCO2. Do not combine with fentanyl or morphine.

Paramedic analgesic-dose ketamine

Medication	Dose / route	Key safeguard
Ketamine - IV/IO	0.3 mg/kg IV/IO slowly over 10 minutes; maximum 30 mg per dose. May repeat every 20 minutes; maximum three total doses.	Do not give by rapid IV push. Use continuous SpO2 and waveform EtCO2; monitor airway, ventilation, sedation, cardiac rhythm, and hemodynamics.
Ketamine - IN	If vascular access is unavailable: 0.5-1 mg/kg IN once; maximum 100 mg.	Single-dose option. Use the same respiratory, cardiac, and sedation monitoring.

Medication selection and monitoring

• Select one opioid. Do not combine fentanyl, morphine, and hydromorphone. Additional opioid dosing beyond the approved total limits requires Medical Control authorization.

• Do not routinely initiate ketamine and an opioid together. If the initial medication is inadequate, fully reassess airway, ventilation, hemodynamics, sedation, pain, and adverse effects before changing agents.

• Use the lower end of the approved opioid range and careful titration in older/frail patients, obstructive sleep apnea, severe COPD, renal or hepatic impairment, or concurrent alcohol, opioid, benzodiazepine, or other sedative exposure.

• A fall in respiratory rate, increasing sedation, abnormal EtCO2, hypoxemia, or loss of airway protection after opioid requires immediate airway support and assisted ventilation; administer naloxone when indicated.

Ketamine exclusions: known hypersensitivity, uncontrolled hypertension, active severe cardiac ischemia, or active psychosis. Do not routinely combine ketamine with a benzodiazepine.

UP-11 Pain Control - Adult

SAFETY, ADJUNCTS, AND DOCUMENTATION

Adjuncts and adverse-effect response

Medication	Dose / route	Key safeguard
Ondansetron	AEMT/Paramedic: 4 mg IV/IO, IM, or PO.	For nausea/vomiting associated with pain or analgesic administration. Continue the analgesic medication's required monitoring.
Naloxone	Paramedic: after airway support and ventilation, 0.4-2 mg IV/IO/IM; if no vascular access, 2 mg IN. Repeat every 2-3 minutes as needed; maximum cumulative dose 8 mg.	Titrate to adequate ventilation rather than complete reversal of analgesia when clinically safe. Use the overdose protocol if overdose is suspected.

Required assessment and safety checks

• Before medication: review allergies, analgesics already taken or administered, alcohol/sedative exposure, opioid tolerance, sleep apnea/COPD, renal or hepatic impairment, anticoagulant use, GI bleeding, pregnancy, and prior medication adverse responses.

• Do not give oral medication when airway protection or swallowing is impaired, active vomiting is present, or another aspiration risk exists.

• Do not give an opioid with respiratory depression, inability to protect the airway, severe hypotension, or significant uncorrected hypoxemia.

• Apply cardiac monitoring for every systemic nonoral analgesic. With opioid or ketamine, maintain continuous SpO2 and waveform EtCO2; keep suction and ventilation equipment immediately available.

• Reassess pain, sedation, airway, respiratory rate/effort, SpO2, EtCO2 when used, BP, cardiac rhythm, and adverse effects at least every 5-10 minutes after parenteral medication and at handoff.

Provider permissions

Provider	Authorized UP-11 care
EMT	Nonpharmacologic care and oral acetaminophen or ibuprofen when indicated and safe.
AEMT	All EMT care plus acetaminophen 1 g IV/IO or ketorolac as above when indicated and safe.
Paramedic	All prior care plus opioid, analgesic-dose ketamine, and adjunctive ondansetron/naloxone as described above.

Documentation and related pathway

• Document pain score or descriptive assessment, medication choice, dose, route, time, monitoring, reassessment findings, adverse effects, and response.

• For patients younger than 16 years, do not use UP-11. Use PM-04 Pediatric Pain Control for age 2 through younger than 16 years; contact Medical Control for patients younger than 2 years.

• This document matches the approved app content reconciled October 10, 2026. Final agency release remains subject to Claiborne County EMS release governance.